



File No.: EXP202211971

RESOLUTION No.: R/00378/2023

Considering the complaint filed on November 4, 2022, with this Agency by A.A.A. (hereinafter, the complainant), against DRSMILE IBERIA, S.L. (hereinafter, the respondent), for not having duly attended to their right of access.

After carrying out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: The complainant exercised the right of access to their medical history against the respondent, and their request, as stated by the complainant, did not receive the legally established response. They provide various documentation related to the complaint filed with this Agency and regarding the exercise of the right exercised.

SECOND: In accordance with Article 65.4 of the LOPDGDD, which has provided a mechanism prior to the admission of complaints filed with the AEPD, consisting of transferring them to the Data Protection Delegates designated by the data controllers or processors, for the purposes provided for in Article 37 of the aforementioned regulation, or to them when they have not been designated, the complaint was forwarded to the respondent entity for analysis and to respond to the complainant and this Agency within one month.

THIRD: The result of the transfer procedure indicated in the previous Fact did not allow for understanding the complainant's claims to be satisfied. Consequently, on February 4, 2023, for the purposes provided for in Article 64.1 of the LOPDGDD, the Director of the Spanish Agency for the Protection of Data agreed to admit the complaint for processing and informed the parties that the maximum period to resolve the present procedure, which is understood to have been initiated by said admission agreement, will be six months.

The aforementioned agreement granted the respondent entity a hearing procedure, so that within ten working days it could present the allegations it deemed appropriate. Said entity expresses regret for the delay in the response and provides the complainant's medical history (dental).

FOURTH: After examining the allegations presented by the respondent, they are subject to transfer to the complainant, so that, within fifteen working days, they may formulate any allegations they consider appropriate. The complainant has not submitted any allegations.

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LEGAL GROUNDS

FIRST: The Director of the Spanish Agency for the Protection of Data is competent to resolve this matter, in accordance with the provisions of paragraph 2 of Article 56 in relation to paragraph 1 f) of Article 57, both of Regulation (EU) 2016/679 of the European Parliament and of the Council of April 27, 2016, on the protection of natural persons with regard to the processing of personal data and on the free movement of such data (hereinafter, GDPR); and in Article 47 of the LOPDGDD.

SECOND: In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for the Protection of Data is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among data controllers and processors regarding their obligations, as well as addressing complaints filed by a data subject and investigating the reasons for them.

Correspondingly, Article 31 of the GDPR establishes the obligation of data controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In the case that they have designated a data protection officer, Article 39 of the GDPR assigns to this officer the function of cooperating with said authority.

In accordance with this regulation, prior to the admission of the complaint that gives rise to this procedure, it was forwarded to the responsible entity for analysis, to respond to this Agency within one month, and to prove that it had provided the complainant with the due response, in the event of exercising the rights regulated in Articles 15 to 22 of the GDPR.

This admission agreement determines the opening of the present procedure for failure to attend to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, according to which:

“1. When the procedure refers exclusively to the failure to attend to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679, it will be initiated by an admission agreement, which will be adopted in accordance with the provisions of the following article.

In this case, the period for resolving the procedure will be six months from the date on which the admission agreement was notified to the complainant. After this period, the interested party may consider their complaint accepted.”

The purification of administrative responsibilities within the framework of a sanctioning procedure is not deemed appropriate, whose exceptional nature implies that, whenever possible, preference is given to alternative mechanisms that are supported by current regulations.

It is the exclusive competence of this Agency to assess whether there are administrative responsibilities that should be clarified in a sanctioning procedure and, in

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As a consequence, the decision regarding its initiation does not entail an obligation to initiate a procedure upon any request made by a third party. Such a decision must be based on the existence of elements that justify the initiation of sanctioning activity, circumstances that do not occur in the present case, considering that with the present procedure the guarantees and rights of the claimant are duly restored.

THIRD: The rights of individuals regarding the protection of personal data are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights of access, rectification, deletion, opposition, the right to limit processing, and the right to portability are contemplated. The formal aspects related to the exercise of these rights are established in Article 12 of the GDPR and Article 12 of the LOPDGDD.

Additionally, consideration is given to what is expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which will be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the data subject, and to express its reasons in case it does not intend to comply with such request. The burden of proof regarding compliance with the duty to respond to the request for the exercise of rights made by the affected party falls on the data controller.

The communication addressed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

Regarding the right of access to personal data, in accordance with Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the data controller may request the affected party to specify the "data or processing activities to which the request refers." The right will be considered granted if the data controller provides remote access to the data, being deemed that the request has been attended to (although the data subject may request the information referred to in the aspects provided in Article 15 of the GDPR).

The exercise of this right may be considered repetitive more than once during a period of six months, unless there is a legitimate reason for it.

On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that incurs a disproportionate cost, which must be borne by the affected

party.

FOURTH: In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the data subject has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data."

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Like the other rights of the data subject, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them and being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the personal data have been or will be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the available information about the origin of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed shall not adversely affect the rights and freedoms of others.

The right of access in relation to the medical history is specifically regulated in Article 18 of Law 41/2002, of November 14, Basic Law Regulating Patient Autonomy and Rights and Obligations in Relation to Information and Clinical Documentation (hereinafter LAP), the literal wording of which states:

- "1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical history and to obtain a copy of the data contained therein. Health centers shall regulate the procedure that guarantees the observance of these rights.
2. The patient's right of access to the medical history may also be exercised by duly accredited representation.
3. The patient's right of access to the documentation of the medical history cannot be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the right of the professionals involved in its preparation, who may oppose the right of access with the reservation of their subjective notes.
4. Health centers and individual practitioners shall only facilitate access to the medical history of deceased patients to persons linked to them, for family or factual reasons, unless the deceased expressly prohibited it and this is accredited. In any case, access by a third party to the medical history motivated by a risk to their health shall be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, shall not be provided."

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the medical history:

- "1. The medical history shall include the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in the field of primary care and specialized care.

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2. The medical record shall primarily aim to facilitate healthcare assistance, documenting all data that, in the medical judgment, allows for a truthful and updated understanding of the health status.

The minimum content of the medical record shall be as follows:

- a) Documentation related to the clinical-statistical sheet.
- b) The admission authorization.
- c) The emergency report.
- d) The anamnesis and physical examination.
- e) The evolution.
- f) The medical orders.
- g) The consultation sheet.
- h) The reports of complementary examinations.
- i) The informed consent.
- j) The anesthesia report.
- k) The operating room report or childbirth record.
- l) The pathology report.
- m) The evolution and planning of nursing care.
- n) The therapeutic application of nursing.
- ñ) The constants chart.
- o) The clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) shall only be required in the completion of the medical record when it concerns hospitalization processes or as otherwise provided.

3. The completion of the medical record, in aspects related to direct patient care, shall be the responsibility of the professionals involved.

4. The medical record shall be maintained with criteria of unity and integration, at each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the healthcare professionals of the data of a specific patient in each healthcare process.

Regarding the preservation of the medical record, Article 17 of the LPA, in its points 1 and 5, states that:

“1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original format, for the proper assistance to the patient for the appropriate time in each case and, at a minimum, five years from the date of discharge from each healthcare process...”

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the healthcare documentation they generate.”

FIFTH: In the case analyzed here, the claimant exercised their right of access to the medical record, and the responding party claims to have addressed it throughout the procedure in a delayed manner. However, it does not provide the corresponding proof that it has communicated to the claimant.

Based on the above, considering that the purpose of this procedure is to ensure that the guarantees and rights of the affected parties are duly restored, and given that the responding party has not proven to have addressed the right by communicating it to the claimant, this resolution is favorable.

Having reviewed the cited provisions and others of general application, the Director of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO GRANT the claim made by A.A.A. and to urge DRSMILE IBERIA, S.L. with NIF B88481924, to send to the claimant, within ten working days following the notification of this resolution, a certification stating that it has addressed the right of access exercised by them or justify the receipt before the claimant that their request has been duly addressed or, to deny it with justification indicating the reasons why it is not appropriate to address their request. The actions taken as a result of this Resolution must be communicated to this Agency within the same period. Non-compliance with this resolution could result in the commission of an infringement of Article 83.6 of the GDPR, classified as very serious for the purposes of prescription in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

SECOND: NOTIFY this resolution to A.A.A. and to DRSMILE IBERIA, S.L.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

Against this resolution, which concludes the administrative procedure in accordance with Article 48.6

of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file a contentious-administrative appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and in paragraph 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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